

Front

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

OFLO* 200/400

OFLOXACIN TABLETS 200 mg / 400 mg

Composition

Each film coated tablet contains:
Ofloxacin USP.....200 mg
Sodium Methyl Hydroxybenzoate BP.....0.5 mg
(as preservative)

Each film coated tablet contains:
Ofloxacin USP.....400 mg
Sodium Methyl Hydroxybenzoate BP.....1.0 mg
(as preservative)

Dosage Form:
Tablet, coated film

Description:
Oflo 200
White to yellowish white coloured round biconvex film coated tablets.

Oflo 400
Pale yellow to yellow coloured round, biconvex film coated tablets.

Pharmacodynamics:
Ofloxacin is a quinolone-carboxylic acid derivative with a wide range of antibacterial activity against both Gram-negative and Gram-positive organisms. It inhibits bacterial DNA replication by blocking DNA topoisom-
erases, in particular DNA gyrase.
Therapeutic doses of ofloxacin are devoid of pharmacological effects on the voluntary or autonomic nervous
systems.
Microbiological results indicate that the following pathogens may be regarded as sensitive: *Staphylococcus
aureus* (including methicillin resistant staphylococci), *Staphylococcus epidermidis*, Neisseria species, *Esche-
richia coli*, Citrobacter, Klebsiella, Enterobacter, Hafnia, Proteus (indole-negative and indole-positive strains),
Haemophilus influenzae, Chlamydiae, Legionella, Gardnerella.
Variable sensitivity is shown by *Streptococci*, *Serratia marcescens*, *Pseudomonas aeruginosa* and Mycoplas-
mas. Anaerobic bacteria (e.g. Fusobacterium species, Bacteroides species, Eubacterium species, Peptococci,
Peptostreptococci) are normally resistant.
Ofloxacin is not active against *Treponema pallidum*.

Pharmacokinetics:
Ofloxacin is almost completely absorbed after oral administration. Maximal blood levels occur 1-3 hours after
dosing and the elimination half-life is 4-6 hours. Ofloxacin is primarily excreted unchanged in the urine.
In renal insufficiency the dose should be reduced.
No clinically relevant interactions were seen with food and no interaction was found between ofloxacin and
theophylline.

Indications:
Ofloxacin is a synthetic 4-fluoroquinolone antibacterial agent with bactericidal activity against a wide range
of Gram-negative and Gram-positive organisms. It is indicated for the treatment of the following infections
when caused by sensitive organisms: Upper and lower urinary tract infections; lower respiratory tract infections;
uncomplicated urethral and cervical gonorrhoea; non-gonococcal urethritis and cervicitis, skin and soft tissue
infections.

OFLO should be only used:
- When Pseudomonas is considered AND the patient is allergic to antipseudo-
monal penicillins/cephalosporins;
- For resistant organisms with no other alternative antibiotics available.
Consideration should be given to official guidance on the appropriate use of antibacterial agents”

Recommended Dose:
Usual daily dose is 200-800 mg of ofloxacin (3-6 tabs) divided into 2-3 doses. Up to 400 mg may be as a single
dose, preferably in the morning. Larger doses should be given as two divided doses. Generally, individual doses
are to be given at approximately equal intervals.
Lower respiratory tract infection: 400 mg daily increasing, if necessary, to 400 mg twice a day. Upper Urinary
tract infection: 200-400 mg daily increasing, if necessary, to 400 mg twice a day.
Lower urinary tract infection : 200-400 mg daily.
Non-gonococcal urethritis and cervicitis: 400 mg daily in single or divided doses.
Skin and soft tissues infections: 400 mg twice daily
Duration of treatment is dependent on the severity of the infection and the response to treatment. The usual treat-
ment period is 5-10 days except in uncomplicated gonorrhoea, where a single dose is recommended.
Treatment should not exceed 2 months duration.

Contraindication:
Ofloxacin tablets is contraindicated in persons with a history of hypersensitivity associated with the use of
Ofloxacin or any member of the quinolone group of antimicrobial agents.

Warning and Precautions:
As with other anti-infectives, prolonged use may result in overgrowth of non-susceptible organisms. If wors-
ening infection occurs, or if clinical improvement is not noted within a reasonable period, discontinue use and
institute alternative therapy.
Caution is recommended if the drug is to be used in Psychotic patients or in patients with a history of psychiat-
ric disease. Use Oflo with caution in patients who have exhibited sensitivities to other quinolone antibacterial
agents. Patients being treated with ofloxacin should not be expose themselves unnecessarily to strong sunlight
and should avoid UV rays (sunlamps, solaria).
The use of Ofloxacin should be avoided in patients who have experienced serious adverse reactions in the past
when using fluoroquinolones containing products (see section Adverse Effects/Undesirable Effects). Treatment
of these patients with Ofloxacin should only be initiated in the absence of alternative treatment options and after
careful benefit/risk assessment.

Aortic aneurysm and dissection
Epidemiologic studies report an increased risk of aortic aneurysm and dissection after intake of fluoroquino-
lones, particularly in the older population. Therefore, fluoroquinolones should only be used after careful bene-
fit-risk assessment and after consideration of other therapeutic options in patients with positive family history
of aneurysm disease, or in patients diagnosed with pre-existing aortic aneurysm and/or aortic dissection, or in
presence of other risk factors or conditions predisposing for aortic aneurysm and dissection (e.g. Marfan syn-
drome, vascular Ehlers- Danlos syndrome, Takayasu arteritis, giant cell arteritis, Behcet’s disease, hypertension,
known atherosclerosis).
In case of sudden abdominal, chest or back pain, patients should be advised to immediately consult a physician
in an emergency department.
Prolonged, disabling and potentially irreversible serious adverse drug reactions
Very rare cases of prolonged (continuing months of years), disabling and potentially irreversible serious adverse
drug reactions affecting different, sometimes multiple body systems (musculoskeletal, nervous, psychiatric and
senses) have been reposed in patients receiving fluoroquinolones irrespective of their age and pre-existing risk
factors. Ofloxacin should be discontinued immediately at the first signs or symptoms of any serious adverse
reactions and patients should be advised to contact their prescriber for advice.

Tendinitis and tendon rupture
Tendinitis and tendon rupture (especially but not limited to Achilles tendon), sometimes bilateral, may occur as
early as within 48 hours of starting treatment with fluoroquinolones and have been reported to occur even up to
several months after discontinuation of treatment. The risk of tendinitis and tendon rupture is increased in older
patients (above 60 years of age), with renal impairment, patients with solid organ transplants, and those treated
concurrently with corticosteroids”. Therefore, concomitant use of corticosteroids should be avoided.
At the first sign of tendinitis (e.g. painful swelling, inflammation) the treatment with ofloxacin should be dis-
continued and alternative treatment should be considered. The affected limb(s) should be appropriately treated
(e.g. immobilisation). Corticosteroids should not be used if signs of tendinopathy occur. *[For systemically
administered levofloxacin-containing products, the listing of risk factors should additionally include: “in patients
receiving daily doses of 1000 mg levofloxacin”.]

Peripheral neuropathy
Cases of sensory or sensorimotor polyneuropathy resulting in paraesthesia, hypaesthesia, dysesthesia, or weak-
ness have been reported in patients receiving quinolones and fluoroquinolones. Patients under treatment with
ofloxacin should be advised to inform their doctor and pharmacist prior to continuing treatment if symptoms of
neuropathy such as pain, burning, tingling, numbness, or weakness develop in order to prevent the development
of potentially irreversible condition (see section Adverse Effects/Undesirable Effects).

Exacerbation of myasthenia gravis
Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with
myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator sup-
port have been associated with fluoroquinolones use in persons with myasthenia gravis. Avoid fluoroquinolones
in patients with known history of myasthenia gravis.

Psychiatric reactions
Psychiatric reactions may occur even after the first administration of fluoroquinolones, including Oflo. In rare
cases, depression or psychotic reactions can progress to suicidal ideations/thoughts and self-injurious behaviour,
such as attempted or completed suicide (see section ‘Side effects’). In the event that the patient develops these
reactions, Oflo should be discontinued, and appropriate measures instituted. Caution is recommended if Oflo is
to be used in psychotic patients or in patients with a history of psychiatric disease.

Interactions with Other Medicaments:
Co-administered magnesium/aluminium antacids, sucralfate or iron preparations can reduce absorption.
Therefore, ofloxacin should be taken 2 hours before such preparations.
Prolongation of bleeding time has been reported during concomitant administration of Ofloxacin and Anticoagu-
lants. Ofloxacin may cause as light increase in serum concentrations of glibenclamide administered concurrent-
ly; patients treated with this combination should be closely monitored.
With high doses of quinolones, impairment of excretion and an increase in serum levels may occur when co-
administered with other drugs that undergo renal tubular secretion (e.g. probenecid, cimetidine, frusemide and
methotrexate).
Interactions with laboratory tests: Determination of opiates or porphyrins in urine may give false-positive results

400 mm

120 mm

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during treatment with ofloxacin.
There may be a further lowering of the cerebral seizure threshold when quinolones are given concurrently with other drugs which lower the seizure threshold e.g. theophylline. However, ofloxacin is not thought to cause a pharmacokinetic interaction with theophylline, unlike some other fluoroquinolones.
Further lowering of the cerebral seizure threshold may also occur with certain nonsteroidal anti-inflammatory drugs.

Pregnancy and Lactation:
Use in pregnancy: There have been no adequate and well-controlled studies performed in pregnant women. Since systemic quinolones have been shown to cause arthropathy in immature animals, it is recommended that Ofloxacin be used in pregnant women only if the potential benefit justifies the potential risk to the foetus.

Use during lactation: Because ofloxacin and other quinolones taken systemically are excreted in breast milk, and there is potential for harm to nursing infants, a decision should be made whether to temporarily discontinue nursing or not to administer the drug, taking into account the importance of the drug to the mother.

Side Effects
The overall frequency of adverse reactions from the clinical trial data base is about 7%. The commonest events involved the gastrointestinal system (about 5.0%) and the nervous system (about 2.0%). The following provides a tabulation based on post marketing experience where occasional represents a frequency of 0.1-1.0%, rare <0.1%, very rare <0.01% and isolated cases << 0.01%.

Digestive and liver side effects:
Occasional: Nausea and vomiting, diarrhoea, abdominal pain, gastric symptoms. (Diarrhoea may sometimes be a symptom of enterocolitis which may, in some cases, be haemorrhagic).
Rare: Loss of appetite, increase in liver enzymes and/or bilirubin.
Very rare: cholestatic jaundice, hepatitis or severe liver damage may develop. A particular form of enterocolitis that can occur with antibiotics is pseudomembranous colitis (in most cases due to *Clostridium difficile*). Even if *Clostridium difficile* is only suspected, administration of ofloxacin should be discontinued immediately and appropriate treatment given. Drugs that inhibit peristalsis should not be administered in such cases.

Central nervous system:
Occasional: Headache, dizziness, sleep disorders, restlessness.
Rare: Confusion, nightmares, anxiety, depression, hallucinations and psychotic reactions, drowsiness, unsteady gait and tremor (due to disorders of muscular co- ordination), neuropathy, numbness and paraesthesia or hypoes- thesia, visual disturbances, disturbances of taste and smell (including, in exceptional cases, loss of function), extrapyramidal symptoms.
Very rare: Convulsions, hearing disorders (including, in exceptional cases, loss of hearing).
These reactions have occurred in some patients after the first dose of ofloxacin. In such cases, discontinue treatment immediately.

Psychiatric disorders
Rare: Psychotic disorder (for e.g. Hallucination).
Very Rare: Psychotic behaviour
Frequency not known (cannot be estimated from the available data): Psychotic disorders and depression with self-endangering behavior including suicidal ideation or suicide attempt.

Cardiovascular system:
Tachycardia and a temporary decrease in blood pressure have been reported.
Rare: circulatory collapse (due to pronounced drop in blood pressure).

Haematological side effects:
Very rare: anaemia, leucopenia (including agranulocytosis), thrombocytopenia, pancytopenia. Only in some cases are these due to bone marrow depression. In very rare cases, haemolytic anaemia may develop.

Renal side effects:
Rare: Disturbances of kidney function.
Isolated cases: Acute interstitial nephritis, or an increase in serum creatinine, which may progress to acute renal failure.

Allergic and skin side effects:
Occasional: Skin rash, itching.
Very rare: Rash on exposure to strong sunlight, other severe skin reactions. Hypersensitivity reactions, imme- diate or delayed, usually involving the skin (e.g., erythema multiforme, Stevens-Johnson syndrome, Lyell's syndrome, and vasculitis) may occur. In exceptional circumstances, vasculitis can lead to skin lesions including necrosis and may also involve internal organs. There are rarely other signs of anaphylaxis such as tachycar- dia, fever, dyspnoea, shock, angioneurotic oedema, vasculitic reactions, eosinophilia. In these cases, treatment should be discontinued immediately and where appropriate, supportive treatment given.
Isolated cases: Pneumonitis.

Other side effects:
Rare: Malaise.
Very rare: Excessive rise or fall in blood-sugar levels. Weakness, joint and muscle pains (in isolated cases these may be symptoms of rhabdomyolysis).
Isolated cases: Tendon discomfort, including inflammation and rupture of tendons (e.g., the Achilles tendon) par- ticularly in patients treated concurrently with corticosteroids. In the event of signs of inflammation of a tendon, treatment with Ofloxacin must be halted immediately and appropriate treatment must be initiated for the affected tendon. The possibility cannot be ruled out that ofloxacin may trigger an attack of porphyria in predisposed patients. Except in very rare instances (e.g., isolated cases of smell, taste, and hearing disorders). The adverse effects observed subsided after discontinuation of ofloxacin.

Adverse Reactions/Side Effects
Exacerbation of myasthenia gravis

Post Marketing Experience
Musculoskeletal and connective tissue disorders*
Nervous system disorders*
General disorders and administrative site conditions*
Psychiatric disorders*
Eye disorders*
Ear and labyrinth disorders*

*Very rare cases of prolonged (up to months or years), disabling and potentially irreversible serious drug reac- tions affecting several, sometimes multiple, system organ classes and senses (including reactions such as tendi- nitis, tendon rupture, arthralgia, pain in extremities, gait disturbance, neuropathies associated with paraesthesia, depression, fatigue, memory impairment, sleep disorders and impairment of hearing, vision, taste and smell) have been reported in association with the use of fluoroquinolones in some cases irrespective of pre-existing risk factors (see section Warnings and Precautions).

Symptoms And Treatment of Overdose
The most important signs to be expected following acute overdosage are CNS symptoms such as confusion, dizziness, impairment of consciousness and convulsive seizures, as well as gastrointestinal reactions such as nausea and mucosal erosions.
In the case of overdose steps to remove any unabsorbed ofloxacin e.g., gastric lavage, administration of adsor- bents and sodium sulphate, if possible, during the first 30 minutes, are recommended; antacids are recommended for protection of the gastric mucosa.
Elimination of ofloxacin may be increased by forced diuresis.

Storage Condition:
Store below 30°C. Protect from light and moisture.

Shelf Life:
3 years from the date of manufacture

Therapeutical Code (If any)
J01MA01: OFLOXACIN

Presentation:
Blister strip of 10 x 10's tablets.

Product Registration Holder and Imported by:
UNIMED SDN BHD
No. 53, Jalan Tembaga, SD 5/2B, Bandar Sri Damansara,
52200 Kuala Lumpur, Malaysia.

 **Manufactured by**
UNIQUE PHARMACEUTICAL LABORATORIES
(A Division of J. B. Chemicals & Pharmaceuticals Ltd.)
Plot No. 215-219, G.I.D.C. Industrial Area, Panoli 394 116,
Gujarat, INDIA
*Trade Mark

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